

Technijian

for Javier Romero, MD FACS

Building the technology foundation for your new surgical sub-specialty clinic.

IT Discovery Meeting | Ravi Jain ↔ Javier Romero, MD FACS | Booked via technijian.com

New clinic infrastructure · **EMR connectivity across Cerner, Epic & Kaiser** · **HIPAA from day one**

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AGENDA

What we'd like to cover today

A working discovery conversation, not a pitch. You told us what you need — interfaces and infrastructure — so that's where we'll spend the time.

OUR GOAL TODAY

Understand your new clinic well enough to design its technology right — and leave you with a clear, no-pressure path from today to a fully built, HIPAA-ready opening day.

You bring the clinical vision.

We bring the build plan — and straight answers about what it takes.

1

What we heard from you

New surgical sub-specialty clinic, Cerner + Epic + Kaiser, interfaces and infrastructure — said back to make sure we got it right.

2

What working across three EMR ecosystems means

The two patterns for Cerner / Epic / Kaiser connectivity — and what each asks of the build.

3

The infrastructure blueprint

The six layers every new clinic needs, from internet circuits to backup and continuity.

4

HIPAA from day one

Compliant-by-design: what a brand-new covered entity must stand up — built into the build.

5

Proof, and how pricing works

Real Technijian healthcare engagements, and transparent per-person pricing with no surprises.

6

Next step

A free site walk and clinic technology assessment — then a blueprint and fixed quote.

WHAT WE HEARD

Your situation, said back to you

From your note when you booked this meeting. Today we fill in the details — correct anything we have wrong.

NEW

CLINIC — BUILT FROM THE GROUND UP

3

EMR ECOSYSTEMS · CERNER · EPIC ·
KAISER

2

WORKSTREAMS · INTERFACES +
INFRASTRUCTURE

DAY 1

HIPAA-READY TARGET AT OPENING

WHAT THAT MEANS

A greenfield build — your one chance to do it right once. No legacy systems to untangle. Every choice — network, devices, phones, security — gets made on purpose, sized to your clinical workflow.

Three EMR ecosystems, one practice. Cerner, Epic, and Kaiser portals means working across three separate health-system IT organizations — credentialed access, secured workstations, and clean referral and results flow into your own records.

A surgical practice runs on uptime and compliance. Scheduling, documentation, imaging review, and billing all sit on this foundation — and as a new covered entity, HIPAA safeguards have to be standing on opening day, not bolted on later.

THE CORE IDEA

A clinic that's born digital — built right once

Most practices inherit technology problems and retrofit around them. You get to skip that entirely — if the build is sequenced right.

The retrofit way

(what most practices live with)

- Devices bought ad hoc, one crisis at a time
- Consumer-grade Wi-Fi carrying patient data
- EMR access set up laptop-by-laptop, by whoever's around
- Compliance bolted on after the fact
- Week-one surprises: dead spots, dropped faxes, no backups

Your greenfield

(built on purpose)

- Every component chosen once, to a plan
- Clinical-grade, segmented network from day one
- Hospital access engineered per ecosystem — all three
- HIPAA safeguards designed into the architecture
- Opening day is boring — everything just works

1

The EMR strategy drives the build

How you'll work across Cerner, Epic, and Kaiser — and whether you run your own practice system — shapes the workstations, network, and security underneath. We settle that first.

2

Lead times gate opening day

Internet circuits, hardware orders, and hospital IT access processes each take weeks. The build schedules backward from your opening date — the earlier we start, the calmer it goes.

3

Compliance is cheapest on day one

HIPAA safeguards designed into the architecture cost a fraction of retrofitting them — and your cyber-insurance application will ask for them anyway.

YOUR FIRST ASK — SOFTWARE INTERFACES

What “using Cerner, Epic and Kaiser” means for the build

Two proven patterns — most surgical practices end up with a blend. Discovery decides the mix; the build then makes it seamless.

PATTERN A — WORK INSIDE THEIR SYSTEMS

Hospital portals, engineered

You document and round inside each hospital's own EMR via credentialed community-provider access.

What that takes: per-hospital access clients (Citrix / VDI / web), secured and certified on every workstation; fast dual-monitor setups; identity hygiene and MFA per health system; and one tested login experience in every exam room — not three fights per patient.

PATTERN B — RUN YOUR OWN SYSTEM

Practice EHR/PM + exchange

Your clinic runs its own EHR / practice-management system for office notes, scheduling, and billing — and exchanges with the hospitals.

What that takes: EHR/PM selection support, HL7 / FHIR interfaces and Direct secure messaging where each hospital supports them, e-fax workflows for the rest, plus eRx and lab / imaging results routing into your charts.

EITHER WAY — WHAT TECHNIJIAN OWNS

We drive all three relationships

Hospital IT controls their side. We own yours — and we manage the process with each organization:

- Access / credentialing applications with each health-system IT
- Workstation builds certified against each portal
- Interface and Direct-messaging setup and testing
- One throat to choke when something breaks

Your three ecosystems are three separate IT organizations. Our job: you see patients — we handle the three IT departments.

YOUR SECOND ASK — INFRASTRUCTURE

The new-clinic blueprint — six layers, one build

Everything a surgical sub-specialty clinic stands on. Each layer is sized to your floor plan, headcount, and workflow during discovery.

LAYER 01

Connectivity & network

Business internet + backup circuit, clinical-grade firewall, segmented Wi-Fi (clinical / office / guest), and structured cabling. Circuits have the longest lead time — we order them first.

LAYER 02

Clinical workstations & devices

Exam-room and front-desk workstations, dual monitors for imaging and portal work, document scanners, label and badge printers — specified once, imaged identically.

LAYER 03

Communications

Healthcare-grade VoIP phones with auto-attendant and after-hours routing, HIPAA-suitable e-fax for referrals and records, and answering-service integration.

LAYER 04

Microsoft 365 & identity

Professional email on your domain, MFA on every login, secure internal messaging, and shared calendars — the identity backbone every hospital portal hangs off.

LAYER 05

Security & HIPAA stack

Managed EDR + threat detection on every device, DNS filtering, disk encryption, automatic patching, and email security — the technical safeguards HIPAA expects.

LAYER 06

Backup & continuity

Independent backup of Microsoft 365 and clinic data, rapid workstation recovery, and documented downtime procedures — so a failed device never cancels a clinic day.

COMPLIANT BY DESIGN

HIPAA from day one — built in, not bolted on

A new practice is a new covered entity. These six items have to exist at opening — we build them into the build itself.

01

Security Risk Analysis

The written risk analysis HIPAA requires (45 CFR 164.308) — and the first thing Medicare/MIPS attestation and OCR ask to see. We run it as part of the build.

02

Business Associate Agreements

Every vendor touching PHI needs a BAA — EHR, e-fax, email, answering service, billing service, IT provider. We inventory them and sign ours on day one.

03

Encryption & access control

Disk encryption on every device, MFA on every login, role-based access, and automatic screen locks — the safe-harbor controls that make a lost laptop a non-event.

04

Workforce training

Short security-awareness training and simulated phishing for your staff from the first hire — people are the most-attacked layer in healthcare.

05

Policies & incident response

Right-sized written policies — acceptable use, access, breach response — plus a tested plan for what happens in the first hour of an incident.

06

Evidence & audit trail

Patching, backups, training, and access reviews — documented continuously, so audits, payer reviews, and insurance renewals are a file pull, not a fire drill.

Bonus: these are the same controls your cyber-insurance application will require — one build satisfies both.

PROOF, NOT PROMISES

We run technology for medical practices today

Three real Technijian healthcare engagements — anonymized for confidentiality, references available on request.

MULTI-PRACTICE MEDICAL GROUP

12 months · zero patient-facing downtime

Full platform operations

VoIP phones, a dozen+ Windows servers, endpoints, and Microsoft 365 for a multi-site, HIPAA-regulated medical group — monthly patch discipline with documented evidence for compliance, and 3CX telephony kept reliable for front-desk and clinical staff.

HEALTHCARE PRACTICE · 18 ENDPOINTS

One weekend · clinical fleet upgraded

Windows 11 cutover, zero clinic disruption

Rolled 18 clinical-office workstations from Windows 10 to 11 over a single weekend — driven by cyber-insurance requirements, with a structured second pass for stragglers and preconfigured spares on standby. No patient-hours lost.

MEDICAL PRACTICE · SOFTWARE + AI

70+ hrs · HIPAA-aware development

Modernized a live clinical web app

Updated the UI and database of a live healthcare application, stood up a clean data service, and integrated an AI assistant that auto-generates authorization checklists — all inside a HIPAA-aware development cycle. This is the muscle behind your interface workstream.

Every environment we manage, we either built or rebuilt. A greenfield clinic is the cleaner version of work we do every week.

THE TECHNIJIAN STACK

One partner, five doors — your build starts with three

Each service stands on its own. For a new clinic: the build project plus managed IT and security as the ongoing foundation.

MANAGED HELP DESK & MONITORING

My IT

Help desk + monitoring + patching for every workstation, the network, and Microsoft 365 — one number your staff calls, with guaranteed response.

Start here.

24/7 PROTECTION

My Security

EDR + threat detection on every device, email security, MFA everywhere, and 24/7 monitoring — the HIPAA technical-safeguard layer, managed.

Start here.

INTERFACES & CUSTOM SOFTWARE

My Dev

Hospital portal access engineering, HL7 / FHIR interfaces, and custom software in a HIPAA-aware development cycle — your interface workstream lives here.

Your build project.

THE HIPAA PROGRAM

My Compliance

Security Risk Analysis, BAA inventory, policies, training, and audit-ready evidence — kept current as you hire and grow.

PLAIN-LANGUAGE REPORTING

My Jian

Technijian-built monitoring that watches across your stack and produces a plain monthly report — so you always know it's working.

HOW PRICING WORKS

Transparent building blocks — no surprises

Real list prices from our standard small-practice packages. Your monthly = your headcount × the stack; the one-time build-out is a fixed-scope quote after the free site walk.

WHAT PROTECTS EACH PERSON · PER MONTH

Endpoint security — EDR + threat detection + DNS filtering + patching + remote support	\$26.50
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Email security & anti-phishing (HIPAA-suitable)	\$4.75
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Microsoft 365 cloud backup	\$4.00
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Security-awareness training (simulated phishing)	\$6.00
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Managed help desk & 24/7 monitoring	~\$46
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Per person	≈ \$87 / mo
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+ Email-domain (DMARC/DKIM) protection — whole practice	\$20 / mo
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Optional as compliance needs grow: encrypted email, single sign-on, activity monitoring & data-loss prevention.

HOW YOUR NUMBER GETS BUILT

≈ **\$87**

per person / month — the managed stack

Illustration only: a 10-person clinic ≈ \$890/mo. Swap in your real headcount and that's your monthly.

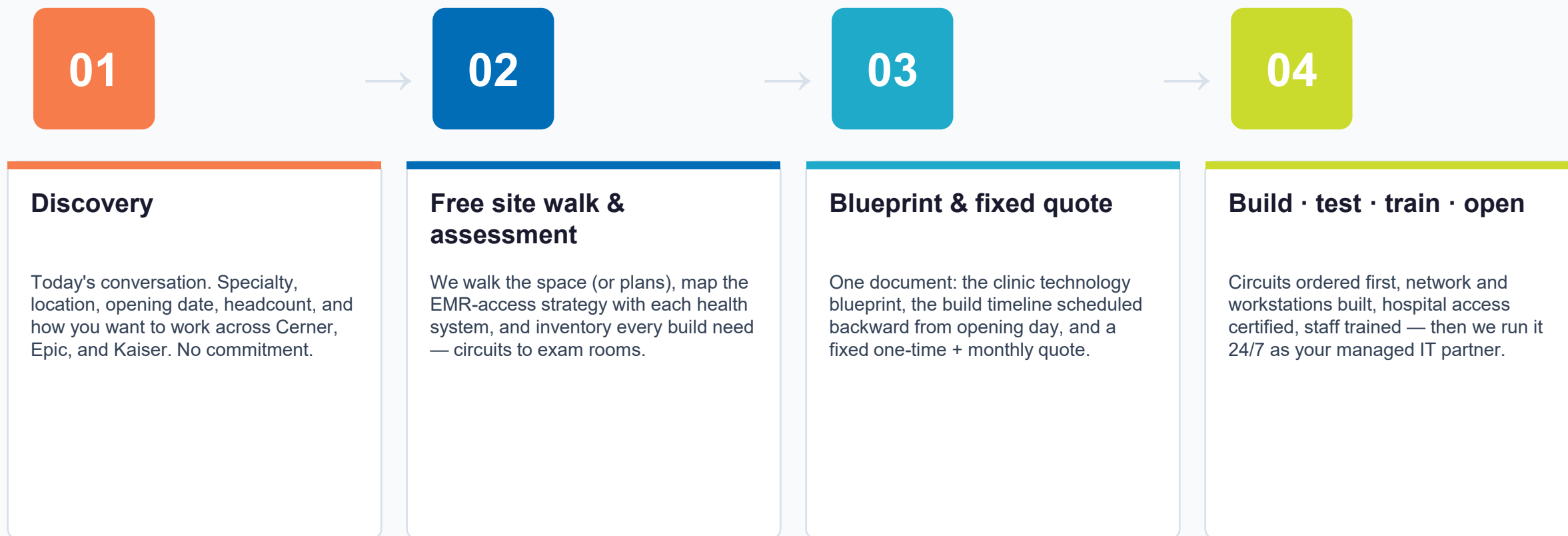
- Flat, predictable monthly — billed per person, every line shown.
- One-time build-out (network, cabling, devices, interfaces): fixed-scope quote after the free site walk.
- Hardware at the configuration you approve — you see and own everything.

Real Technijian list prices from our standard small-practice packages; the 10-person figure is illustrative only. Your fixed monthly and one-time build-out quote follow the free site assessment.

HOW WE'D ENGAGE

Discovery first. A blueprint before a dollar. Then we build.

Every step is gated — you see the plan and the fixed quote before committing to anything.



Timing note: internet circuits and hospital IT access processes are the long poles — engaging 90+ days before opening keeps everything off the critical path.

NEXT STEP

One next step. **No commitment.**

RECOMMENDED

Free site walk & clinic technology assessment

We walk your space or plans, map the access strategy with Cerner, Epic, and Kaiser, and come back with the clinic technology blueprint and a fixed quote — one-time build plus monthly. You decide from there.

YOU KEEP THIS EITHER WAY

The New Clinic IT Checklist (1 page)

A plain-language checklist of everything a new practice needs — circuits, devices, HIPAA items, and the lead times for each — yours to keep and act on, whether or not we work together.

Book your site walk · rjain@technijian.com · 949.379.8499 · technijian.com